



Policy

Emergency Psychiatric Treatment

Policy code	EMHS: 18
Original endorsement date	July 2016
Date of current endorsement	July 2022
Version	2.0
Functional subgroup	Mental Health
Summary	Definition: Emergency psychiatric treatment (EPT) in the Mental Health Act 2014 section 202 means treatment that needs to be provided to a person: to save the person's life; or prevent the person from behaving in a way that is likely to result in serious physical injury to the person or another person
Policy owner	Executive Director Mental Health
Review date	July 2025
ISD Impact Statement Declaration	The consultation through the Aboriginal community is reflected in this policy in this policy to ensure cultural appropriateness and positive health impacts on the Aboriginal community (ISD Record ID)
NSMHS Standards	2.3, 2.12.2, 13, 6.5, 6.7, 7, 8.10, 9.3, 9.5
Status	Active

1. Background

1.1. About this document

The East Metropolitan Health Service (EMHS) will provide emergency psychiatric treatment where indicated and will carry this out in accordance with relevant legislation, government policy, standards of practice and guidelines.

Definition: Emergency psychiatric treatment (EPT) in the [Mental Health Act 2014](#) section 202 means treatment that needs to be provided to a person:

- to save the person's life; or
- to prevent the person from behaving in a way that is likely to result in serious physical injury to the person or another person.

Emergency psychiatric treatment does not include electroconvulsive therapy, psychosurgery, deep sleep therapy, insulin coma therapy or insulin sub coma therapy.

Refer to [Mental Health Act 2014](#) Sections 202, 203 and 204 and Part 5, Div 2 (consent issues)

Refer to [Clinicians Practice Guide to the Mental Health Act 2014](#) (June 2024)

[MP0175/22 WA Health Consent to Treatment Policy](#)

2. Emergency Psychiatric Treatment (EPT)

The administration of EPT must comply with Sections 202, 203 and 204 of the Mental Health Act 2014 Office of the Chief Psychiatrist EPT treatment

Refer to the [Office of the Chief Psychiatrist EPT treatment](#)

Refer to the [Clinicians Practice Guide to the Mental Health Act 2014](#) (June 2024) and Supplement

Refer to the Department of Health, [Mental Health Act 2014](#) Referral Process under the section 25 Div 2 Referrals for examination for further information.

3. Consent Issues

Referred persons can be offered treatment and if they give their informed consent, it is not deemed to be EPT.

However, if a referred person refuses treatment or is unable to consent because of their mental state, and they meet the criteria for EPT, they can be provided with treatment without consent.

A clinician must seek valid consent from a person before providing psychiatric treatment. Consent is valid if freely and voluntarily given and a failure to offer resistance to treatment does not of itself constitute consent to treatment.

Before consent is sought the patient must be given a clear explanation of the proposed treatment including sufficient information such as warnings of risks and what is known or not known about the effects of the treatment

Information should be:

- that which will assist the patient in making a balanced judgement about whether or not to consent to the treatment.
- sufficient for a reasonable person in the patient's position to regard as significant and include information that the person is likely to consider personally important.
- conveyed in a form that would enable the patient to readily understand the issues, including the use of information in other languages or the use of interpreters.
- provided with sufficient time for the patient to consider the information, including whether to seek advice and assistance from other sources.

EPT must only be given to save the person's life or to prevent serious physical injury to the person or another person. EPT should not commence merely because the clinician believes it would be advantageous to commence treatment before the person is examined or made an involuntary patient.

EPT does not include regulated treatments such as ECT, psychosurgery, deep sleep therapy, insulin coma therapy or insulin sub coma therapy or regulated treatments such as seclusion or restraint.

EPT can also be given to any voluntary patient who has not been referred, e.g. a patient in an ED or any hospital who becomes behaviourally disturbed and meets the criteria for EPT.

Refer to [MP0175/22 WA Health Consent to Treatment Policy](#) and [Mental Health Act 2014](#) section 203.

4. Personal Support Persons

4.1. Personal Support Persons

Wherever possible personal support persons (meaning carers, close family members or other personal support persons) should be engaged in treatment decisions.

Refer to WA. [Carers Recognition Act 2004](#)

Refer to EMHS [Rights of Carers and other Personal Support Persons](#)

Refer to [Mental-Health-Act-2014-Summary-for-Carers](#)

5. Restraint

If the referral occurs within an authorised hospital, and the person needs to be restrained in order for the treatment to be given, that is a bodily restraint and the Restraint Form 9A [Emergency Psychiatric Forms](#) (password protected) must be completed. Please note this is not required for a restraint external to an authorised hospital.

Any use of mechanical bodily restraint must be authorised and may be used in an emergency in accordance with Mental Health Act 2014 section 232.

Any use of restraint must be in accordance with the EMHS Restraint and Seclusion CPS

Refer to [Mental Health Act 2014](#) Sections 231, 232, and 233.

6. Documentation

6.1. Documentation

A medical practitioner who provides EPT (Emergency Psychiatric Treatment) must, as soon as is practicable, record that EPT was provided and complete an EPT Form 9A . The form will include:

- The name of the referred person provided with EPT
- The name and qualifications of the practitioner who provided the treatment
- The names of any other people involved in provided the treatment including staff who may have been involved in restraining the
- Person for the treatment to be given.
- The date, time, and place of when the treatment was provided
- Particulars of the circumstances of why the EPT was required
- Particulars of the actual treatment provided, such as the type of medication

A copy of the Form 9A must be given to the referred person and a copy sent to the Chief Psychiatrist.

If the person is a mentally impaired accused, a copy is to be given to the Mentally Impaired Accused Review Board.

Staff in the EMHS Mental Health who give EPT are to file a record of the provision of the treatment and give a copy to the patient, the Chief Psychiatrist and (if applicable) the Mentally Impaired Accused Review Board.

The record is to meet the requirements of section 204(2) of the Mental Health Act 2014. A template of the EPT reporting form is available from the Office of the Chief Psychiatrist (OCP).

If restraint is used to provide EPT, a notation about this is to be made on the record.

7. Compliance monitoring and legislative penalties

7.1. Compliance monitoring

Each EMHS Site/Service Executive Director are responsible for ensuring staff in their area/department are made aware of and comply with this policy, and that compliance risks and training requirements are managed.

Staff are to be reminded that compliance with all policies is mandatory.

7.2. Legislative obligations

There are legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

8. Legislation

- Mental Health Act 2014, sections 202-204, 232.
- Mental Health Regulations 2015
- WA Carers Recognition Act 2004

9. Additional references

9.1 WA Health policies

- Mental Health Act 2024

9.2 EMHS policies

- Consent Policy

9.3 Others

- Consent to Treatment Policy for the Western Australian Health System
- Emergency Psychiatric Treatment and Issues of Consent
- SMHS Seclusion in Mental Health Policy and Appendix A Restraint and / or Seclusion: Management of Patients Exhibiting Disturbed, Aggressive or Violent Behaviour in Inpatient Mental Health Units.
- SMHS Restraint of Mental Health Patients Policy
- DoH Office of the Chief Psychiatrist Clinician's Guide to the Mental Health Act 1996, fifth edition – 2011 and its Supplement 2011.

- DoH Office of the Chief Psychiatrist Medical Practitioners' Guide to the Referral Process under the Mental Health Act 1996